A doctor-in-training learns bedside manners, the hard way

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The 16-inch metal rod in my leg and the seven-inch surgical scar descending down my abdomen remind me daily of how much has changed in my life in the last year.

But it wasn't until last month, when I returned to medical school and the anatomy lab, that how fortunate I am truly started to sink in.

Like all medical students, I spend a lot of time getting to know human anatomy both through books and by dissecting a cadaver. One day, early this semester, I reached into the cadaver's pelvis and grasped the iliac artery, as thick as a taproot, carrying blood from the heart to vital organs such as the bladder, and to muscles of the backside.

I held it in awe. This is one of the arteries that I ruptured in my own pelvis, less than a year ago.

Another day, I explored how several muscles converged to attach on the cadaver's upper leg.

Then I felt my own hip, and noticed differences with these muscles in my own still-healing body.

"Wow, no wonder it still hurts," I thought, reflecting on the pain I feel when I stand too long.

When I started medical school in August 2015, I felt overwhelmed from the first day. The number of muscles, bones, nerves, arteries, and veins that I needed to memorize seemed, well, inhuman. Who knew there were 27 bones in each hand?

After bombing the first exam, I doubled down on studying, and did well on the second test about six weeks later. After studying during all of the next Saturday, I felt comfortable enough to take a late-afternoon break to drive to visit my girlfriend, a senior at my alma mater, Lafayette College, about an hour from Temple's campus in North Philadelphia.

But I never got there. As I was making a left turn from a stop sign onto Route 309, a car slammed into my driver's side door at 55 mph.

Four days later, I woke up in the ICU at Lehigh Valley Hospital with no memory of what had happened. I was told that I nearly died and needed two operations in the first 24 hours to stop internal bleeding and to repair multiple pelvic fractures and a broken femur.

When I came out of the induced coma, I was attached to a ventilator with a breathing tube down my throat and a cervical collar on my neck. I could neither speak nor turn my head. I was so grateful to see my parents, who had driven overnight from North Carolina, at my bedside, along with my girlfriend.

This was the beginning of a new and unexpected part of my medical education, with lessons I believe will make me a better doctor.

First, I learned the power of reaching out to help someone in need. I was blown away by the love and support I received in visits, notes, and calls.

Former professors, old college friends, new classmates I hardly knew, and friends and colleagues of my parents' appeared at my bedside. Kind words, particularly from people I didn't even know that well, had a powerful and undeniable impact on my mindset and recovery. No matter how much pain I was in, I tried to act as if I felt better so my visitors would feel comfortable sitting with me. After a while, I realized that doing this really made me feel better.

I hope I'll always remember how helpless and alone a patient can feel — especially those without friends or family — and that the smallest kindness I can offer may make a big difference.

Second, I learned the value of teamwork in medicine in a way I'll never forget.

One example is what happened when it came time to wean me off the ventilator. To prove I was strong enough to breathe on my own, I needed to breathe for 30 minutes with the tube still in, but with the machine turned off. Breathing through a ventilator tube feels like breathing through a straw, and was unbelievably hard for someone who hadn't used his breathing muscles for days.

The respiratory team was suddenly called away because a patient in the next room was in cardiac arrest. So there I was, breathing through the tube for more than an hour. I was dripping with sweat, and it took serious coaching from family and the excellent nurses who stepped in to keep me afloat until the respiratory experts returned. Seeing how the team all trusted each other and worked together, however, gave me the confidence I needed to get through.

The third major lesson came after I left the hospital. While recovering, I took multiple daily walks through the neighborhood with my grandfather, whose health was starting to deteriorate from lung cancer. A 22-year-old on crutches escorted around the block by an 88-year-old was quite the sight!

My grandfather and I enjoyed each other's company tremendously, and I listened to his wisdom more intently than ever before. One pearl: "Anything can be fixed

after a few hours tinkering in my workshop" — in other words, keep on working to solve problems. Never give up.

My grandfather died a few months later. Now, a year after my accident, I am driven, in part by his memory, back to where I started, as a first-year medical student in anatomy.

Whether I'm sorting out tangled nerves in the neck that resemble spaghetti or discovering that veins feel so much softer than arteries, my perspective has been forever transformed. The smell of formaldehyde hasn't improved from last year, but my score on the first anatomy exam sure did!

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